METROPOLITAN HEART INSTITUTE
CARDIOLOGY CLINICAL DISCHARGE RECORD

PATIENT DETAILS:

Patient Name: Rajesh Varma | Age: 59 | Gender: Male
UHID: MHI-CARD-2026-9041 | Admission Date: 2026-09-20 | Discharge Date: 2026-09-24
Attending Cardiologist: Dr. Arvind Deshmukh, MD, DM (Cardiology)

CHIEF COMPLAINTS & PRESENTATION:

Patient presented with retrosternal crushing chest pain radiating to left shoulder and jaw, accompanied by diaphoresis and shortness of b

PRIMARY DIAGNOSIS:

1. Acute ST-Elevation Myocardial Infarction (Anterior STEMI).
2. Coronary Artery Disease (Single Vessel Disease - 95% proximal LAD occlusion).
3. Primary Dyslipidemia.

HOSPITAL INTERVENTION:

Emergency Primary Percutaneous Coronary Intervention (PCI) with Drug-Eluting Stent (DES, 3.0 x 24 mm) deployed to Proximal Left Ant

CRITICAL LAB RESULTS & INVESTIGATIONS:

- High-Sensitivity Troponin-I: Peak 4,200 ng/L (Post-procedure down to 180 ng/L)
- CK-MB: 88 IU/L (Elevated)
- Total Cholesterol: 245 mg/dL | LDL: 168 mg/dL | HDL: 34 mg/dL | Triglycerides: 215 mg/dL
- HbA1c: 5.8% (Non-diabetic range)
- Serum Creatinine: 1.0 mg/dL (Normal renal clearance)
- 2D Echocardiogram: Anterior wall hypokinesis, Left Ventricular Ejection Fraction (LVEF): 46%.

DISCHARGE PRESCRIPTION & MEDICATION REGIMEN:

1. Tab. Aspirin 75 mg - 1 tablet orally once daily after lunch (Antiplatelet).
2. Tab. Ticagrelor 90 mg - 1 tablet orally twice daily (Morning and Night) - DUAL ANTIPLATELET THERAPY (DAPT).
3. Tab. Atorvastatin 80 mg - 1 tablet orally daily at bedtime (High-intensity lipid lowering).
4. Tab. Metoprolol Succinate ER 25 mg - 1 tablet orally once daily morning (Beta-blocker for cardioprotection).
5. Tab. Ramipril 2.5 mg - 1 tablet orally once daily at bedtime (ACE inhibitor for ventricular remodeling).
6. Tab. Pantoprazole 40 mg - 1 tablet orally once daily 30 minutes before breakfast (Gastric protection).

CRITICAL MEDICATION ALERTS & CONTRAINDICATIONS:

- ABSOLUTELY ESSENTIAL: Do NOT miss or discontinue Ticagrelor or Aspirin without direct cardiology authorization. Stopping DAPT prematur
- BLEEDING PRECAUTIONS: Avoid NSAIDs (Ibuprofen, Naproxen). For mild headaches/pain, use Paracetamol only.
- Watch for red-flag bleeding: Black tarry stools, dark urine, or persistent unprovoked nosebleeds.

PATIENT RECOVERY & LIFESTYLE GUIDANCE:

- Cardiac Rehabilitation: 20-30 minutes light flat-surface walking daily. Strictly avoid heavy weight lifting (>5 kg) for 4 weeks.
- Dietary Plan: Mediterranean cardiac diet. Eliminate trans fats, reduce saturated fats, low sodium (<2 g/day).
- Strictly avoid grapefruit juice (interferes with statin and antiplatelet metabolism).
- Emergency Red Flags: Immediate ER visit if recurrence of retrosternal heaviness, cold sweats, or sudden fainting.

FOLLOW-UP SCHEDULE:
Review in Cardiology OPD in 3 weeks with fasting lipid profile and resting 12-lead ECG.